

Patient ID: EGPatient ID: EGPeriod: March 15 to May 6, 2024Report Date: May 7, 2024Coverage: Bed: 915/1245h (73.5%)Chair: 670/1245h (53.8%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 53 days from Mar 15 to May 06

HRBreathing

		#3	#3		#4		#5		
Mar 15		Mar 29		Apr 12		Apr 26			

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
39	45h	High Breathing	<1	100%

Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	High Breathing	46 brpm	2h	1h	<1	18 brpm	Apr 25 to Apr 27
2	High Breathing	32 brpm	1h	1h	1	19 brpm	May 05
3	Elevated Breathing	40 brpm	14h	2h	1	16 brpm	Mar 27 to Apr 04
4	Elevated Breathing	46 brpm	14h	2h	2	19 brpm	Apr 11 to Apr 17
5	Elevated Breathing	46 brpm	7h	1h	1	18 brpm	Apr 20 to Apr 26
6	Elevated Breathing	46 brpm	2h	1h	2	19 brpm	May 05

RR values outside physiologic range (6–60 breaths/min).

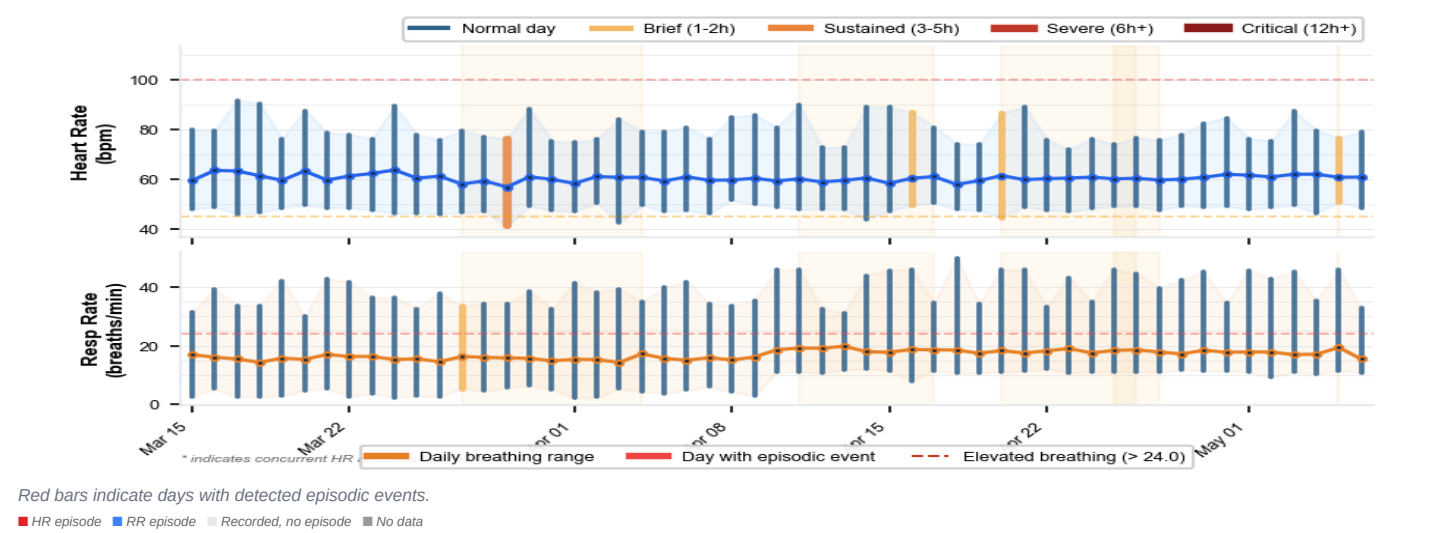
Clinical Pattern Observations:

1. Clustered pattern:

Episodic events on 5 of 53 days (9%).

2. Daytime pattern:

Episodes concentrated during waking hours.



Clinical Alerting Thresholds

Very Low HR < 40 bpm

Low HR < 45 bpm

Elevated HR > 95 bpm

High HR > 100 bpm

Very High HR > 110 bpm

Elevated Breathing > 24 brpm

High Breathing > 30 brpm

Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.